

ADR Identification Basics

VirtualWorks Internship — Task 1 | ID: task1

1. Patient Scenario

Patient	Female, 45 years
Presenting symptoms	Rash, itching, nausea and dizziness
Medication history	Amoxicillin (started 5 days ago); Paracetamol as needed
Timeline	Amoxicillin started on Day -5 → symptoms began on Day -2 → symptoms persisted on Day 0

2. ADR Assessment

Is the reaction potentially drug-related? Yes. The symptoms began approximately three days after amoxicillin was started, giving a plausible temporal relationship. Rash and itching are also recognized types of adverse reactions associated with amoxicillin. Nausea and dizziness may also occur with medication use, although other causes should be considered.

3. Suspected Drug

Amoxicillin is the primary suspected drug because it was newly started before the symptoms appeared. Paracetamol was used only as needed and is therefore less temporally suggestive from the information provided.

4. Evidence Supporting the Association

Factor	Observation
Temporal relationship	Drug started Day -5; symptoms began Day -2.
Clinical pattern	Rash and itching are compatible with a drug-related hypersensitivity-type reaction.
Alternative medicines	Paracetamol was used as needed; no stronger temporal link is provided.
Assessment	The case supports amoxicillin as the suspected medicine, but causality cannot be proven from the information provided.

5. Final Conclusion

Suspected ADR: Yes.

Suspected drug: Amoxicillin.

The symptoms have a plausible temporal association with newly initiated amoxicillin. A complete clinical assessment would be required to establish causality and severity.

Prepared as an educational internship-task submission based on the patient scenario displayed in the VirtualWorks task preview.